

--SUMMARY--

Decision No. 542/21

17-Aug-2021

E.Kosmidis - K.Soden - D.Broadbent

- Second Injury and Enhancement Fund {SIEF} (severity of accident)
- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)
- Accommodation (modified duties)

The worker, a cashier, tripped and fell in October 2015, fracturing her toe and ankle. In 2016, she was diagnosed with Pain Disorder and Adjustment Disorder. The worker returned to work as a cashier in March 2018 and was provided accommodation of a chair to sit in while performing her duties. She stopped working on March 26, 2018. The worker appealed a decision of the Appeals Resolution Officer find that the accommodated work was suitable and she was not entitled to LOE benefits. The employer appealed a decision granting SIEF relief of 25% for a minor pre-existing condition and a moderate accident.

The employer's appeal was dismissed and the worker's appeal was allowed.

The accident was of moderate severity. Recent Tribunal decisions have drawn a distinction between a slip, which does not result in a fall to the ground, and a slip and fall. There appeared to be a general consensus in the Tribunal decisions that a slip alone would be considered a minor accident, while a slip and fall can be expected to cause a disabling injury and was of moderate severity. The determination of accident severity was still case-specific. In this case, the worker was throwing garbage into a bin when she tripped on uneven concrete. The act of throwing the garbage created some momentum when falling to the ground. The accident would be expected to cause a disabling injury.

The worker's pre-existing psychological condition was minor. She was being treated by her family doctor with medication, and her problems did not prevent her from working.

The modified duties were unsuitable, as it was not feasible for the worker to sit while performing cashier duties. She also experienced difficulties while performing her other assigned duties of stocking shelves and sweeping. The worker was entitled to LOE benefits.

13 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w4021s]
- BOARD DIRECTIVES AND GUIDELINES: Operational Policy Manual, Document No. 14-05-03
- TRIBUNAL DECISIONS CONSIDERED: Decision No. 865/14, 2014 ONWSIAT 1055 refd to, Decision No. 159/20, 2020 ONWSIAT 273 refd to, Decision No. 1012/20, 2020 ONWSIAT 1660 refd to

Style of Cause:
Neutral Citation: 2021 ONWSIAT 1288



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 542/21

BEFORE:

E. Kosmidis : Vice-Chair
K.J. Soden : Member Representative of Employers
D. Broadbent : Member Representative of Workers

HEARING:

April 14, 2021 at Toronto
Oral by Teleconference

DATE OF DECISION:

August 17, 2021

NEUTRAL CITATION:

2021 ONWSIAT 1288

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) dated February 26, 2020
and May 14, 2020

APPEARANCES:

For the worker:

C. Topple, Paralegal

For the employer:

B. Lisson, Paralegal

Interpreter:

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REASONS

(i) Introduction/Issues

[1] The worker appeals a decision of the ARO dated February 26, 2020.

[2] The employer appeals a decision of the ARO dated May 14, 2020.

[3] The issues under appeal are as follows:

ARO February 26, 2020

1. The suitability of the accommodated pre-accident employment;
2. Whether the worker is entitled to full LOE benefits from March 26, 2018.

ARO May 14, 2020

3. The employer seeks an increase in the quantum of SIEF cost relief currently at 25%.

(ii) Background

[4] According to the Form 6 dated June 20, 2016, the now 63 year old worker was employed as a cashier in a retail store. The worker tripped and fell on October 7, 2015. She was seen at the emergency department where she was diagnosed with a displaced transverse intra-articular fracture through the base of the fifth metatarsal along with a tiny avulsion fracture from the top of the medial malleolus.

[5] Dr. Kaminker, the orthopaedic surgeon who assessed the worker at the hospital, recommended non-operative treatment with the use of an aircast. However, a January 28, 2016 x-ray report showed nonunion of the fracture at the base of the left fifth metatarsal.

[6] The worker was referred to a Regional Evaluation Centre (REC) where she was assessed by Dr. Santone, orthopaedic surgeon. In his report dated January 28, 2016, he diagnosed the worker with a nonunion of the fifth metatarsal fracture. He recommended surgery in the form of open reduction, internal fixation and possible bone grafting of the worker's fifth metatarsal fracture. Following surgery, the post-operative protocol would include a plaster cast for two weeks and non-weight bearing followed by fiberglass cast for an additional four weeks and non-weight bearing. The report outlined the worker's further recovery as follows:

At her 6-week follow-up visit, we would take off the fiberglass cast and place her into an AirCast, at which point she can begin to touch weight bear. She would also be able to begin physiotherapy for ankle range of motion without any strengthening exercises at that time.

We would then progress her to weight bearing as tolerated from week 7 to 8 with the AirCast and the crutches. Weight bearing as tolerated with the AirCast and no crutches from week 8 to 10.

[7] From week 10 to 12, she would be able to weight bear as tolerated, without the AirCast or crutches.

The above plan is dependent on the post-operative x-rays showing excellent signs of healing and union

- [8] Dr. Santone also recommended a Concurrent Mood Assessment in order to address the worker's increasing depression. He opined that this treatment would assist with her depressive symptoms as well as provide strategies for pain management.
- [9] Dr. Oosterhoff, Psychologist, and Dr. Melnyk, Psychiatrist, conducted the Concurrent Mood and Anxiety Assessment on March 2, 2016. In their report dated March 11, 2016, they opined that the worker did not need treatment in the Concurrent Treatment Program. Based on the worker's presentation and report, they opined that her mood and pain symptoms did not appear to be acutely problematic at that time. They noted that the worker reported managing reasonably well at this point with family support and was unsure whether psychological intervention would be beneficial. Therefore, they concluded that the worker did not need psychological treatment at this time.
- [10] The surgery was performed by Dr. Santone on March 17, 2016. In his Operative Follow up Report dated March 31, 2016, he notes that the worker's sutures and her cast were removed and she was fitted with a fiberglass cast and informed to continue non-weight bearing for the next four weeks.
- [11] The worker began a post surgical rehabilitation program on May 6, 2016 to assist with functional ability and return to work.
- [12] In his report dated June 23, 2016, Dr. Santone states that the June 23, 2016 x-ray report shows a complete union of the base of the fifth metatarsal. He recommended that the worker receive an additional eight weeks of post-surgical rehabilitation. However, the worker was discharged from the rehabilitation program due to poor attendance with her last treatment on August 23, 2016.
- [13] The worker also underwent a second mood and anxiety assessment on June 23, 2016. The worker was diagnosed with Pain Disorder with psychological factors and a general medical condition, chronic Adjustment Disorder with depressed mood.
- [14] The worker saw Dr. Santone on October 23, 2017. Dr. Santone opined that the worker had the following restrictions:
- Functional Restrictions & Return to Work Recommendations
- The worker is able to complete modified duties at a PDC level of Sedentary, with walking and standing limited to an occasional basis. A more detailed description of her functional restrictions will be determined upon completion of the Functional Abilities Evaluation.
- [15] He opines that the worker may have reached MMR but it is anticipated that the Medical Specialty Pain Consultation will allow her to gain some pain relief from the nerve discomfort that she is currently exhibiting. He recommended that the worker attend a Medical Specialty Pain Consultation for further treatment recommendations.
- [16] In his January 28, 2018 report, Dr. Santone opines that the worker is able to perform seated sedentary duties, with walking and standing limited to an occasional basis. She should have the opportunity to take seated breaks as needed to help manage her symptoms. Her current functional restrictions will be more clearly outlined upon completion of the FAE.
- [17] The worker underwent a Functional Abilities Evaluation. The February 7, 2018 report notes that the worker self-limited several activities making it difficult to determine her true functional restrictions to aid in return to work. However based on what the worker was willing

to perform, her overall performance fell in the sedentary category of work. The report describes sedentary level work as primarily involving sitting, but may also involve an occasional amount of brief walking and standing in order to carry out job duties.

[18] The worker returned to work in March of 2018 to an accommodated cashier position. The worker stopped working on March 26, 2018. In a decision dated March 8, 2019, the Board determined that the worker was not entitled to LOE benefits as the accommodated work offered by the accident employer was suitable. The worker's appeal of this decision was denied by the ARO in a decision dated February 26, 2020.

[19] The employer requested SIEF cost relief in a letter dated March 7, 2018. In a decision dated June 15, 2018 the employer's request was denied. However, in a decision dated September 28, 2018, the employer was granted 25% cost relief on the basis of a minor pre-existing condition with a moderate accident severity. The Case Manager noted that the worker was receiving treatment and medication for her non-organic issues prior to the date of accident. The medical evidence substantiated that the worker's medication for her non-organic issues had been increased after her surgery. The Case Manager determined that "it is reasonable to assume that her pre-existing issues made her more susceptible to developing a psychological issue."

[20] The employer's appeal of this issue was denied by the ARO in a decision dated May 14, 2020.

[21] Both ARO decisions have been appealed to this Tribunal.

(iii) Law and policy

[22] Since the worker was injured in 2015, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[23] Specifically, sections 40 and 43 of the WSIA govern the worker's entitlement in this case. Section 40 of the WSIA provides in part:

40(1) The employer of an injured worker shall co-operate in the early and safe return to work of the worker by,

- (a) contacting the worker as soon as possible after the injury occurs and maintaining communication throughout the period of the worker's recovery and impairment;
- (b) attempting to provide suitable employment that is available and consistent with the worker's functional abilities and that, when possible, restores the worker's pre-injury earnings;
- (c) giving the Board such information as the Board may request concerning the worker's return to work; and
- (d) doing such other things as may be prescribed. 1997, c. 16, Sched. A, s. 40 (1).

(2) The worker shall co-operate in his or her early and safe return to work by,

- (a) contacting his or her employer as soon as possible after the injury occurs and maintaining communication throughout the period of the worker's recovery and impairment;
- (b) assisting the employer, as may be required or requested, to identify suitable employment that is available and consistent with the worker's functional abilities and that, when possible, restores his or her pre-injury earnings;

- (c) giving the Board such information as the Board may request concerning the worker's return to work; and
- (d) doing such other things as may be prescribed. 1997, c. 16, Sched. A, s. 40 (2).

...

[24]

Section 43 of the WSIA provides in part that:

43(1) A worker who has a loss of earnings as a result of the injury is entitled to payments under this section beginning when the loss of earnings begins. The payments continue until the earliest of,

- (a) the day on which the worker's loss of earnings ceases;
- (b) the day on which the worker reaches 65 years of age, if the worker was less than 63 years of age on the date of the injury;
- (c) two years after the date of the injury, if the worker was 63 years of age or older on the date of the injury;
- (d) the day on which the worker is no longer impaired as a result of the injury. 1997, c. 16, Sched. A, s. 43 (1).

...

(3) The amount of the payment is 85 per cent of the difference between his or her net average earnings before the injury and any net average earnings the worker earns after the injury, if the worker is co-operating in health care measures and,

- (a) his or her early and safe return to work; or
- (b) all aspects of a labour market re-entry assessment or plan. 1997, c. 16, Sched. A, s. 43 (3); 2000, c. 26, Sched. I, s. 1 (6).

(4) The Board shall determine the worker's earnings after the injury to be the earnings that the worker is able to earn from the employment or business that is suitable for the worker under section 42 and is available and,

- (a) if the worker is provided with a labour market re-entry plan, the earnings shall be determined as of the date the worker completes the plan; or
- (b) if the Board determines that the worker does not require a labour market re-entry plan, the earnings shall be determined as of the date the Board makes the decision. 2007, c. 7, Sched. 41, s. 2 (2).

...

(7) The Board may reduce or suspend payments to the worker during any period when the worker is not co-operating,

- (a) in health care measures;
- (b) in his or her early and safe return to work; or
- (c) in all aspects of a labour market re-entry assessment or plan provided to the worker. 1997, c. 16, Sched. A, s. 43 (7).

[25]

As noted above, one of the issues before the Tribunal is the worker's entitlement to LOE benefits. Under section 43(1) a worker who has a loss of earnings as a result of a compensable injury is entitled to LOE benefits. *Decision No. 2474/00* held that under section 43(1) a causal relationship between the injury and wage loss is a condition precedent to the payment of LOE benefits. A refusal of suitable work is not necessarily an act of non-cooperation, but it may lead to a conclusion that the worker's loss of earnings does not result from the injury. Section 43(2)

operates to reduce a worker's benefits where the worker refuses suitable employment. Thus, a worker who refuses suitable employment at no wage loss is not entitled to LOE benefits because the loss of earnings is not caused by the injury, but caused by the refusal of the suitable employment.

[26] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[27] The standard of proof in workers' compensation proceedings is the balance of probabilities. Pursuant to subsection 124(2) of the WSIA, the benefit of the doubt is resolved in favour of the claimant where it is impracticable to decide an issue because the evidence for and against the issue is approximately equal in weight.

[28] Pursuant to section 126 of the WSIA, the Board provided the policies applicable to this appeal. We have considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual* (OPM) Document No. 14-05-03, "Second Injury end Enhancement Fund (SIEF)";
- OPM Document No. 19-02-01, "Work Reintegration Principles, Concepts and Definitions".

[29] OPM Document No. 18-03-02 "Payment and Reviewing LOE Benefits (Prior to Final LOE Review)," explains the circumstances in which "Treatment with No Return to Work" is appropriate:

If the nature or seriousness of the injury completely prevents a worker from returning to any type of work, the worker is entitled to full LOE benefits, providing the worker co-operates in health care measures as recommended by the attending health care practitioner and approved by the WSIB. If the worker does not co-operate, the WSIB may reduce or suspend the worker's LOE benefits.

(iv) Analysis

The suitability of the accommodated pre-accident employment

[30] The worker's representative submitted that while the employer tried to accommodate the worker's restrictions, the work that was offered to the worker was unsuitable and unsustainable. He referred to the worker's testimony noting that she tried to work as a cashier in a seated position but that the worker testified that this was unfeasible and aggravated her left ankle and foot.

[31] The employer's representative submitted that the worker's left foot injury was not totally disabling noting that the worker had only been awarded a 1% NEL for this injury. The employer's representative submitted that the return to work process requires the cooperation of both parties but in this case, while the employer provided suitable accommodated work, the worker never returned to work.

[32] The employer's representative submitted that the Board and the employer went to extraordinary lengths to get the worker back to work and held return to work meetings. He also noted that the worker was also having knee problems which he submitted were a factor in the worker's failure to return to work. Finally, the employer's representative submitted that the

worker's family doctor's opinion should be given little weight as he assumed the role of an advocate and did not have a full understanding of the issues.

[33] After carefully considering the worker's testimony, the medical evidence and the submissions from both representatives, the worker's appeal of this issue is allowed. The Panel finds that the accommodated work offered to the worker was unsuitable for the reasons set out below.

[34] The worker testified that she began working as a cashier for the accident employer, a large retail chain, in 2005. She worked 30 hours per week. As a cashier, she was required to stand.

[35] After her October 7, 2015 left foot injury, she returned to work after six weeks. She was assigned work that involved putting labels on store items, at reduced hours. She was stationed in the kitchen/lunchroom area. She had to use pain killers to deal with her pain. The pain made her depressed.

[36] The worker testified that as her left foot condition did not improve, she had an x-ray that showed that her foot was not healing. She went back to the doctor who told her to wear a cast and that she could not go back to work for a period of six weeks. When she went back to the doctor after six weeks in the cast, she was told that she needed surgery which was performed on March 17, 2016.

[37] The worker testified that after her surgery, she could not stand on her own and her leg was in a bandage. The doctor told her that she would be further assessed after six weeks. However, the worker testified that one week after her surgery she received a call from the WSIB and was told that she had to attend a meeting at the store. She advised the WSIB that she could not attend this meeting.

[38] She went back to her doctor who put her foot in a cast again for another six weeks. During this time she used a wheel chair and a walker to get around. After her cast was removed, she was told that she could wear shoes and that she could go back to work.

[39] The worker testified that she returned to work as a cashier. The worker testified that she was in a great deal of pain and could not stand. She was told by her doctor that the nerves in her leg were damaged and that she needed a second surgery.

[40] The worker testified that she was given a chair to sit in while performing her cashier duties. However, since the store was so busy, the worker testified that she could not use the chair consistently.

[41] The worker testified that there were problems with the chair in that it was higher than the cash register and that when the cash register was open, it hit the chair. It was not possible to sit in the chair and handle transactions or wrap merchandise. Additionally, if she sat in the chair, the cash drawer would also hit her knee.

[42] The worker testified that she was assigned the busiest store hours despite the fact that she asked her manager to schedule her when the store was less busy.

[43] The worker testified that in addition to cashier duties, she was also assigned duties consisting of climbing a ladder to stock shelves and sweeping duties at the end of the afternoon shift. The sweeping involved the entire store and took 15 to 20 minutes. Her foot was very sore when she finished her shift.

- [44] The worker testified that her right knee became symptomatic after her left ankle surgery as she was forced to put more weight on her right leg.
- [45] The worker testified that that she stopped working and became depressed requiring her to take stronger medication. The worker testified that she has not looked for work since she stopped working for the accident employer. She testified that she is unable to stand as she has nerve damage.
- [46] The worker testified that she had right knee replacement surgery in 2019 and that this has improved her knee pain and she is able to stand on her right leg. She is also getting her left knee replaced but the date has not been set. The worker has not looked for work since she stopped working at the accident employer.
- [47] In his October 23, 2017 report, Dr. Santone opined that the worker required modified duties at the sedentary level with walking and standing on an occasional basis.
- [48] In his report dated January 18, 2018, Dr. Santone records that the worker is complaining of constant aching pain that begins at the distal aspect of the dorsolateral left foot and travels along the lateral foot, and up the left fibula. Aggravating factors include prolonged standing for up to 30-45 minutes, prolonged walking for 20 minutes, and sitting with the foot in a gravity-dependent position. The pain is eased with rest and the use of naproxen.
- [49] The worker reported to Dr. Santone that she stopped working in September 2017, as the modified duties that were offered to her were not appropriate. She reported that, although she was provided with a stool to sit on, she was required to stand for the majority of the day and could not sit, as she was regularly scheduled to work during busy hours. She reported that she was unable to sit and work at the cash register at the same time.
- [50] Dr. Santone recommended that the worker have permanent restrictions. He opined that the worker was able to perform seated sedentary duties, with walking and standing limited to an occasional basis. She should have the opportunity to take seated breaks as needed to help manage her symptoms. However, he stated that her current functional restrictions would be more clearly outlined upon completion of the Functional Abilities Evaluation (FAE).
- [51] The worker underwent a FAE on February 7, 2018. The report from this FAE records that the worker was able to perform standing work on an occasional basis and that she demonstrated the ability to walk on an occasional basis provided it is self-paced.
- [52] The Panel accepts the opinion of Dr. Santone which is supported by the FAE and finds that the worker has permanent restrictions limiting her ability to stand and walk to an occasional basis.
- [53] The Panel also accepts the worker's uncontradicted testimony as to the duties that she was assigned when she returned to work in March of 2018. The Panel finds that in addition to her cashier duties, she was also assigned stocking shelves which required the use of a ladder and sweeping the entire store. The Panel accepts the worker's testimony that she experienced difficulties performing these duties.
- [54] The Panel accepts the worker's testimony that she was unable to perform her cashier duties while seated. The Panel agrees with the submissions from the worker's representative that it was not feasible to have a cashier fully sit and perform her duties of cashing out and bagging purchases while sitting. The worker testified that her hours of work were some of the busiest

times for the store and that it was not possible for her to sit and cash out customers when the store was so busy.

[55] The worker testified that the chair provided was also problematic in that the chair was higher than the cash register and when the cash drawer was open, the drawer would hit the chair making it impossible to sit in the chair and handle transactions. This required the worker to stand to work as a cashier. The Panel also finds that the duties of sweeping and using a ladder to stock shelves also exceeded the worker's restrictions. Accordingly, the Panel finds that the modified duties of cashier were not suitable.

[56] The worker's appeal of this issue is allowed.

Whether the worker is entitled to full LOE benefits from March 26, 2018

[57] The Panel has found that the accommodated cashier job offered to the worker was unsuitable. The worker's representative submitted that the issue of the worker's ongoing entitlement to LOE should be referred back to the Board.

[58] The worker testified that she is still an employee of the accident employer. She started her employment with the accident employer in 2007 and has worked exclusively as a cashier since that time. The Panel notes that the worker is now 64 years old with restrictions that limits her standing and walking to an occasional basis. Therefore, the Panel finds that the worker has limited transferable skills and significant restrictions. The Panel also notes that the worker has continued to pursue treatment for her condition and that the clinical notes confirm that she continued to see her family doctor discussing various treatments and medication for her psychological condition and the ongoing pain in her left foot. As such, the Panel finds that the worker was unlikely to find employment without the assistance of the Board.

[59] As the worker's loss of earnings was related to her compensable left foot injury, the Panel finds that the worker is entitled to full LOE benefits from March 26, 2018.

[60] The extent of the worker's ongoing entitlement to LOE is referred back to the Board for further determination, subject to the usual rights of appeal.

[61] The worker's appeal of this issue is allowed.

Increase in SIEF cost relief

[62] The Board's authority to establish the Second Injury and Enhancement Fund derives from section 98 of the WSIA, which states:

98 (1) The Board may establish a special reserve fund to meet losses that may arise from a disaster or other circumstance that, in the opinion of the Board, would unfairly burden the employers in any class.

[63] The amount of SIEF relief granted is dependent on two variables: the severity of the accident and the severity of the pre-existing condition. The Board determined that the worker's accident was of moderate severity with a minor pre-existing condition, thereby resulting in 25% SIEF cost relief.

[64] With respect to the matter of the pre-existing condition, Board policy requires that it be characterized as minor, moderate or major. The policy does not define these terms and indicates only that the medical significance of a condition is assessed "in terms of the extent that it makes

the worker liable to develop a disability of greater severity than a normal person”. These provisions were interpreted as follows in *Decision No. 1582/07*:

I interpret the policy to mean that the medical significance of a pre-existing condition should be considered to be “minor” if it made the worker slightly more liable to develop a disability of greater severity than a normal person, and that it should be considered “major” if it made the worker extremely liable to develop a disability of greater severity than a normal person. If the extent to which the pre-existing condition made the worker more liable to develop a disability of greater severity than a normal person was more than slight, but less than extreme, the medical significance of the pre-existing condition could be considered moderate.

[65] Under OPM Document No. 14-05-03, entitlement to SIEF relief is available in two circumstances. First, SIEF relief may be granted if “a prior disability caused or contributed to the compensable accident.” Second, SIEF relief is available “if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition. Both “pre-accident disability” and “pre-existing condition” are defined terms under this policy. Therefore, when read together, Board policy authorizes entitlement to SIEF relief when:

- The worker had a pre-accident disability which required treatment and disrupted employment pre-accident, and that disability causes or contributes to the accident, or
- The worker had an underlying or asymptomatic condition which becomes manifest post- accident, and that condition prolongs or enhances the recovery or claims period.

[66] The employer's representative submitted the worker had a pre-existing noncompensable knee condition that prolonged the worker's recovery in this claim. The employer's representative further submitted the worker's pre-existing psychological condition also prolonged the worker's recovery. The worker's representative submitted the worker was already diagnosed with anxiety and depression and had to increase her medication for her psychological condition because of her left ankle fracture. The employer's representative submitted that the SIEF relief of 25% should be increased.

[67] The employer's appeal of this issue is denied for the following reasons.

[68] There is no evidence that the worker had a prior disability that caused or contributed to the compensable accident. Therefore, the issue in this case is whether the worker had a pre-existing condition that prolonged the worker's recovery period.

[69] The Panel finds that the worker's underlying noncompensable knee arthritis was not a pre-existing condition that enhanced or prolonged the worker's recovery in this claim. I rely on the opinion of Dr. S. Wentzell, the Board's Medical Consultant who was asked to review the worker's medical records in order to provide an opinion on whether the worker's pre-existing noncompensable bilateral knee arthritis was evidence of an underlying or pre-existing condition that enhanced or prolonged the claim.

[70] In Memo #132 dated June 11, 2018, Dr. Wentzell reviewed the worker's medical records and opined that while the worker had arthritis in her knees and had been offered a right total knee replacement, the worker had not yet had the right knee replacement surgery. He opined that despite the worker's pre-existing arthritis in her knees, it was highly likely that the worker's recovery trajectory for the left foot would have remained the same regardless of her knee arthritis.

- [71] The Panel was not directed to any opinion that contradicted Dr. Wentzell's opinion. The Panel accepts Dr. Wentzell's opinion and finds that the worker's pre-existing noncompensable knee arthritis did not enhance or prolong the worker's recovery in this claim.
- [72] On the issue of the worker's psychological condition, the Panel has reviewed Dr. Ireland's clinical notes. The worker reported emotional distress on March 24, 2014 and she was prescribed Restoril. On August 14, 2015, the worker reported that she was having difficulty with a manager who was overbearing and very critical. The worker had complained about this manager but her complaints had not been addressed. The worker was in tears and unable to sleep. Dr. Ireland prescribed Lorazepam.
- [73] The worker testified that she was already taking medication when she broke her ankle but that her depression increased after her surgery as she realized that she was not going to be pain free. The worker testified that her depression got worse after her injury.
- [74] The Panel also observes that the WSIB Foot & Ankle Specialty Clinic Comprehensive Assessment Report of January 28, 2016 provides a Clinical Screening Questionnaire and Psychosocial Status. Of note, the worker's Pain Catastrophizing Scale (PCS) score "suggests presence of significant psychological factors which may impact the worker's perception of pain & disability." This was also discussed in the Concurrent Mood and Anxiety Assessment Report of March 11, 2016, with yellow flags being extreme symptom reporting, high pain levels, fear and avoidance of activity, fear of re-injury and pain catastrophizing. Accordingly and as also found by the Board, the Panel is satisfied the worker's pre-existing psychological condition likely prolonged or enhanced the worker's recovery from the workplace injury.
- [75] The Panel did not hear any submissions on the severity of the pre-existing condition. However, the Panel notes that the worker's pre-existing psychological problems did not prevent her from working. The worker was being treated by her family doctor with medication. There is no evidence before this Panel that Dr. Ireland referred the worker to a specialist for any further treatment prior to the October 7, 2015 accident. This is confirmed by Dr. Ireland in his letter dated November 1, 2018.
- [76] When all of this evidence is considered, the Panel finds that the worker's pre-existing condition was of minor severity.
- [77] The Panel did not hear any submission relating to the severity of the accident. Based on its review of the evidence, the Panel finds that the accident was of moderate severity. The Panel notes that a more recent line of Tribunal decisions regarding slip and fall accidents in the context of SIEF related appeals, have determined that these types of accidents are of moderate severity. These decisions draw a distinction between a slip, which does not result in a fall to the ground, and a slip and fall type accident where the worker actually falls to the ground. These decisions conclude that the former type of accident, which is a more common or frequent occurrence, can be considered a minor accident expecting to cause a non-disabling or minor disabling injury. With respect to the latter, there appears to be a general consensus that a slip and fall injury can be expected to cause a disabling injury, recognizing that these types of injuries may vary significantly, from no injuries at all to death. See for example *Decision Nos. 159/20* and *2012/20*.
- [78] Similarly, *Decision No. 865/14* determined a slip and fall accident to be of moderate severity. The worker, in that case, fell forward on her knees and her left upper extremity.

[79] Although there appears to be a general consensus among the more recent Tribunal decisions, I find that a determination of accident severity is still very much case specific. This view is reflected in some recent Tribunal decisions that have determined that a slip and fall accident, based on the circumstances that arose in those particular cases, were of minor severity. In those cases, the analysis has included a consideration of the surrounding circumstances, such as the terrain and the speed in which the worker was moving.

[80] The worker was throwing garbage into the garbage bin with a co-worker when she tripped over uneven concrete and fell to the ground twisting her ankle. Based on the mechanics of the accident the Panel finds that the act of throwing the garbage created some momentum when falling to the ground. The Panel finds that the accident severity was moderate as it would be expected to cause a disabling injury.

[81] According to the chart, a minor pre-existing condition with a moderate accident severity results in a 25% SIEF relief for the employer.

[82] The employer's appeal is denied.

DISPOSITION

[83]

The appeal is allowed in part as follows:

1. The accommodated cashier job is unsuitable.
2. The worker is entitled to full LOE under section 43 from March 26, 2018. The duration of the worker's ongoing entitlement to LOE benefits is referred back to the Board for further determination, subject to the usual rights of appeal.
3. The employer's appeal seeking an increase in SIEF cost relief is denied.

The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: August 17, 2021

SIGNED: E. Kosmidis, K.J. Soden, D. Broadbent